• Patients should provide their full name, date of birth, contact number and the requested specialty.

• Patients should bring a valid identification document and relevant previous medical reports when available.

• Arrive approximately 15 minutes before a scheduled outpatient appointment for registration.

• Appointment times are estimates and may change because of emergencies or clinical delays.

• Patients who need to cancel or reschedule should contact the appointment desk as early as possible.

• An appointment confirmation does not guarantee that a particular doctor will be available if an unforeseen clinical event occurs.

Fictional appointment line: +92 300 555 0122.

6. Registration and Patient Identification

At registration, the hospital creates or verifies a patient record. Accurate identification is important because records, laboratory results, medications and clinical instructions must be associated with the correct person.

• Patients should provide accurate identifying information.

• Staff may verify identity using more than one identifier before clinical procedures, medication administration, specimen collection or other high-risk activities.

• Patients should inform staff if their name, address, telephone number, emergency contact or other registration information changes.

• Patients should not use another person’s hospital account or identification.

• Parents or authorized guardians provide information for patients who cannot complete registration themselves.

7. Visiting and Family Information

Area	Visiting hours	Additional information
General inpatient wards	4:00 PM–7:00 PM daily	Maximum 2 visitors per patient at a time
ICU	5:00 PM–6:00 PM daily	Maximum 1 visitor at a time; clinical circumstances may restrict visits
Pediatrics	4:00 PM–7:00 PM daily	Parent/guardian arrangements may differ
Maternity	4:00 PM–7:00 PM daily	Visitor limits may apply for privacy and infection control
Emergency Department	Case-by-case	Access may be restricted during treatment, procedures or emergencies

Visitors should perform hand hygiene, follow staff instructions, avoid visiting when unwell and respect patient privacy. Visiting rules may be temporarily changed during outbreaks, emergencies or other safety events.

8. Inpatient Admission

Admission may occur after assessment in an outpatient clinic, Emergency Department or another clinical service. The admitting team determines whether inpatient care is appropriate.

• Registration and identification are completed or verified.

• The clinical team records relevant medical history, current medications and allergies as available.

• Patients receive information about their room or ward, basic routines, communication channels and safety expectations.

• Valuables should not be brought unnecessarily. The hospital is not responsible for unsecured personal valuables.

• Patients should tell staff about allergies, mobility limitations, communication needs and other safety concerns.

• Discharge planning may begin early during admission, particularly when follow-up, medication changes or home support will be needed.

9. Discharge and Follow-Up

Before discharge, the clinical team reviews the patient’s condition and determines whether discharge is appropriate.

• Patients should receive discharge instructions appropriate to their condition.

• Medication instructions should be reviewed before leaving.

• Follow-up appointments or referral instructions should be provided when required.

• Patients should understand which symptoms require urgent medical attention.

• Patients should ask the clinical team questions if instructions are unclear.

• Copies of relevant reports may be provided according to hospital procedure and applicable privacy rules.

The AI assistant may explain general discharge information contained in this document but must not replace a clinician’s individualized discharge instructions.

10. Diagnostic Laboratory

The laboratory provides routine and urgent diagnostic testing. Test availability depends on the requested investigation, equipment, staffing and clinical circumstances.

Service	Typical availability
Routine blood testing	Mon–Sat, 7:00 AM–8:00 PM
Urine testing	Mon–Sat, 7:00 AM–8:00 PM
Urgent inpatient testing	Available according to clinical need
Emergency laboratory support	24 hours
Result communication	Through the treating clinical team or approved patient-access channel

Patients may be asked to follow preparation instructions such as fasting for selected tests. The required preparation depends on the specific test; patients should follow the instructions provided by their healthcare professional.

11. Radiology and Imaging

Radiology provides diagnostic imaging including X-ray, ultrasound and CT. Some examinations require specific preparation.

• Scheduled imaging generally operates from 8:00 AM to 8:00 PM, Monday through Saturday.

• Emergency imaging is available 24 hours according to clinical need.

• Patients should tell staff if they may be pregnant before an examination involving ionizing radiation.

• Patients should report known allergies or previous reactions when contrast material may be considered.

• Patients should follow instructions regarding food, fluids, medications or other preparation when provided.

12. Pharmacy

The hospital pharmacy supports inpatient and emergency services and provides outpatient prescription dispensing during its stated operating hours.

• Prescription medicines are dispensed according to authorized prescriptions.

• Patients should tell pharmacy or clinical staff about allergies and medicines or supplements they are already taking.

• Patients should not share prescription medicines with other people.

• Patients should follow the label and clinician instructions and should ask a pharmacist or clinician if directions are unclear.

• Medication availability can change; an equivalent or alternative may require clinician approval.

• Emergency and inpatient pharmacy support is available 24 hours according to hospital operational arrangements.

13. Infection Prevention and Hygiene

The hospital follows standard infection-prevention principles for patient care. WHO identifies hand hygiene as a major component of standard precautions and emphasizes risk-based use of personal protective equipment. ■cite■turn0search4■turn0search5■

• Patients, visitors and staff should perform hand hygiene at appropriate times.

• Visitors should not enter patient-care areas when they have symptoms of a potentially contagious illness unless instructed otherwise by clinical staff.

• Respiratory hygiene and cough etiquette should be followed.

• Additional precautions may be required for patients with suspected or confirmed infectious conditions.

• Shared clinical equipment should be cleaned and disinfected according to applicable procedures.

• Staff use personal protective equipment according to the anticipated exposure risk and applicable clinical precautions.

These principles are included as general information. They do not replace current infection-control instructions issued by the hospital infection-prevention team. WHO describes infection prevention and control as essential to protecting patients and health workers from avoidable infections. ■cite■turn0search1■turn0search2■

14. Patient Rights

Patients have the right to respectful and safe care. The following fictional hospital principles are aligned conceptually with international patient-safety principles; they are not a statement of legal rights in any jurisdiction. WHO’s Patient Safety Rights Charter describes patient rights in the context of safe healthcare. ■cite■turn0search3■

• To be treated with dignity and respect.

• To receive understandable information about their care within applicable professional and legal limits.

• To participate in decisions about their care when clinically appropriate.

• To have reasonable privacy during care.

• To have health information handled confidentially subject to applicable law and authorized use.

• To raise concerns or complaints without retaliation.

• To request clarification when instructions are unclear.

• To receive care without discrimination based on protected characteristics under applicable law.

15. Patient Responsibilities

• Provide accurate health and registration information.

• Tell healthcare workers about known allergies and current medicines.

• Follow reasonable safety instructions from hospital staff.

• Treat patients, visitors and staff respectfully.

• Keep appointments or notify the hospital when cancellation is necessary.

• Maintain confidentiality and privacy of other patients.

• Do not bring weapons, illegal substances or hazardous items into the hospital.

• Ask questions when medical or administrative instructions are unclear.

16. Safety and Fall Prevention

Patients with weakness, dizziness, mobility limitations or other fall risks should tell staff. Staff may apply additional precautions based on assessment.

• Use assistance when staff recommend it.

• Use handrails and mobility aids when provided.

• Keep walkways clear of bags and personal items.

• Report spills, damaged equipment or other hazards immediately.

• Do not adjust medical equipment unless instructed.

• Children should remain supervised by an appropriate adult.

Fall-prevention measures are individualized; this section does not provide a clinical fall-risk assessment.

17. Fire and Evacuation Safety

• Remain calm and follow instructions from hospital staff or emergency personnel.

• Do not use elevators during a fire evacuation unless specifically instructed by emergency personnel.

• Do not block corridors, exits, stairways or fire equipment.

• Visitors should stay with or follow the staff member responsible for directing them during an evacuation.

• Do not return to an evacuated area until authorized.

Emergency exits and assembly arrangements are displayed in hospital areas. Staff receive emergency preparedness training according to hospital policy.

18. Security and Prohibited Conduct

• Violence, threats, harassment and intimidation are prohibited.

• Weapons and dangerous items are prohibited except where specifically authorized for security or emergency personnel.

• Smoking and vaping are prohibited inside hospital buildings and designated restricted areas.

• Alcohol and illegal drugs may not be brought into or used on hospital premises except medicines administered or handled through authorized clinical processes.

• Photography, recording or live streaming involving patients or clinical areas requires appropriate permission and must respect privacy.

• Security staff may restrict access to areas that are not open to the public.

19. Privacy and Confidentiality

Patient information is confidential and should only be accessed, used or disclosed for authorized purposes. The exact legal requirements depend on the applicable jurisdiction and hospital policy.

• Staff should access only information needed for their authorized duties.

• Patients should avoid sharing passwords or account credentials.

• Printed records should be protected from unauthorized access.

• Requests for records or information may require identity verification and authorization.

• The AI assistant must not disclose private patient records merely because a user asks for them.

20. Complaints, Feedback and Incident Reporting

Patients and visitors may report concerns about care, communication, facilities, billing or other hospital services.

Channel	Purpose
Patient Relations Desk	Questions, complaints and service concerns
Nursing supervisor	Immediate ward-level concerns
Quality & Safety Office	Patient-safety concerns and incident reporting
Billing Office	Charges, invoices and payment questions
Security Desk	Safety, lost property and security concerns

For urgent safety concerns, notify the nearest staff member immediately rather than waiting for a formal complaint process.

21. Billing and Insurance

The fictional hospital accepts self-pay patients and selected insurance arrangements. Coverage depends on the insurance plan and authorization requirements.

• Patients should verify insurance eligibility before planned services when possible.

• Some services may require prior authorization.

• An estimate is not necessarily the final bill.

• Questions about charges should be directed to the Billing Office.

• Emergency care is handled according to clinical priority; financial or insurance questions should not delay notification of urgent clinical needs.

22. Accessibility and Communication Support

• Patients should tell registration or clinical staff about accessibility or communication needs.

• Reasonable assistance may be arranged according to available hospital resources.

• Patients with hearing, vision, mobility or communication needs may request appropriate support.

• Where interpretation is required, staff should use approved interpretation arrangements rather than relying on children or untrained companions for important clinical communication.

23. Food, Nutrition and Dietary Services

Inpatients may receive meals according to their prescribed or documented dietary requirements. Dietary restrictions, allergies and culturally appropriate needs should be communicated to the clinical or nutrition team.

• Patients should inform staff about food allergies.

• Outside food may be restricted in some clinical areas or for certain patients.

• Patients should follow clinician instructions regarding fasting before procedures or tests.

• Nutrition counseling is available by referral during stated service hours.

24. Transportation and Parking

• The hospital provides a designated patient drop-off area near the main entrance.

• Emergency vehicles use the emergency entrance and must have unobstructed access.

• Visitors should park only in designated areas.

• Vehicles should not block fire lanes, ambulance routes or emergency entrances.

• Patients requiring medical transport should discuss arrangements with the clinical team before discharge.

25. Hospital AI Assistant Information Policy

The hospital AI assistant is a software component of this demonstration project. It retrieves information from approved knowledge documents and uses a language model to formulate responses.

• The AI assistant is intended for general hospital information and navigation, not diagnosis or individualized treatment.

• Retrieved information should be treated as the hospital knowledge source for the application.

• If the retrieved documents do not contain an answer, the assistant should state that the information is unavailable rather than inventing a policy.

• Emergency symptoms should be directed to emergency services or qualified clinical staff.

• Private patient information should not be disclosed without appropriate authorization.

• Answers generated by the AI assistant should not override instructions from a treating clinician or hospital staff.

• Document version and source metadata should be retained so the application can identify which knowledge was used.

This section is particularly important for RAG testing because the desired behavior is **grounded answering**: the model should answer from retrieved hospital information and avoid unsupported claims.

26. RAG Knowledge-Base Metadata

The document is designed for ingestion into a retrieval-augmented generation system. Each extracted chunk should retain source metadata.

Metadata field	Example
document_id	CCGH-INFO-001
document_title	CityCare General Hospital – Hospital Information Guide
document_version	1.0
source_type	hospital_information
department	general
effective_date	2026-10-08
page	Generated PDF page number
chunk_index	Sequential chunk number
access_level	public_hospital_information

Recommended metadata fields for the RAG implementation include document ID, filename, document type, department, page number, section title, chunk index, version and effective date. Metadata allows the retriever and application to identify where a retrieved answer came from.

27. Frequently Asked Information

Question	Expected answer from this document
Is the Emergency Department open 24 hours?	Yes. The Emergency Department is open 24 hours a day, seven days a week.
What are general ward visiting hours?	4:00 PM to 7:00 PM daily, with a maximum of two visitors at a time.
What are ICU visiting hours?	5:00 PM to 6:00 PM daily, with a maximum of one visitor at a time.
What is the appointment phone number?	+92 300 555 0122 (fictional).
What is the main hospital number?	+92 300 555 0100 (fictional).
When is the Cardiology Department open?	Monday through Saturday, 9:00 AM to 5:00 PM.
Is emergency radiology available?	Yes, emergency imaging is available 24 hours according to clinical need.
Can the AI assistant diagnose me?	No. It is intended for general hospital information and navigation, not diagnosis or individualized treatm
What should I do during a fire?	Remain calm, follow staff/emergency instructions, use designated evacuation routes and do not use el
Can I request my patient information?	Requests may be made through approved hospital processes and may require identity verification and

28. Example RAG Test Questions

These questions are included to help test retrieval quality after ingestion. They are not additional hospital policy.

- What time can I visit someone in the ICU?
- Is the emergency department open on Sunday?
- What is the cardiology department schedule?
- What number do I call to make an appointment?
- What should visitors do if they are feeling sick?
- Where should I go for urgent medical problems?
- What information should I bring for registration?
- What services does the pharmacy provide?
- When is routine laboratory testing available?
- What should I do if I need wheelchair or communication assistance?
- Can the AI assistant give me a diagnosis?
- How do I report a safety concern?
- What are the general inpatient visiting hours?
- What are the hospital's rules about smoking?
- How does the hospital protect patient privacy?

29. Document Governance

Field	Value
Document ID	CCGH-INFO-001

Field	Value
Version	1.0
Created	08 October 2026
Review cycle	For demonstration: review whenever source information changes
Owner	CityCare General Hospital Information Management (fictional)
Status	Active for software demonstration
Supersedes	Initial version

When a production hospital changes its hours, telephone numbers, services or policies, the corresponding knowledge document should be updated and re-indexed. Old versions should not remain retrievable as current information unless versioning and effective dates are explicitly handled.

30. Disclaimer

This entire document describes a fictional hospital created for development and RAG testing. It is not an actual hospital policy manual and does not establish legal, medical, billing, insurance or regulatory requirements.

The safety and infection-prevention concepts included here were informed by publicly available WHO patient-safety and infection-prevention resources, including WHO standard precautions, hand-hygiene guidance, patient-safety rights, and infection-prevention programme guidance.
■cite■turn0search1■turn0search3■turn0search4■turn0search5■

For a real hospital deployment, every operational fact and clinical policy must be approved by the hospital's authorized clinical, legal, privacy, quality and administrative teams before being placed into a production knowledge base.